



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : فاتن حمامه محمد شحاته ممم

العنوان : مم

الحي / المدينة : الزيتون

المحافظة : القاهرة

تاريخ تسجيل الإستمارة : 23-09-2026

رقم البطاقة : 26004242300444 رقم التليفون : 01110471005

العمر (تاريخ الميلاد) : 24-04-1960 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي : (.....) التوقيع: (.....)

المقرر بما فيه : (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج الالم ومضادات القئ لمرضي

قسم العلاج :
الاورام

الاورام المنتشر [14456]

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني : الوظيفة : : التوقيع :
الاسم : د/ندى حسن : الوظيفة : : التوقيع :
الاسم : د/نرمين رمضان : الوظيفة : : التوقيع :

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... /د. /..... /د. /..... /د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : : قسم :
جهة العلاج : : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: فأتن حمامه محمد شحاته مميم National ID: 26004242300444	
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تقرير طبي

السيد
تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حمید بالثدى و تحتاج الى Pin Medication And Anti Emetics
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية

فاتن حمامه
محمد شحاته
هش احمد عبدالعاطي
مركز اوسيم - الجيزة

١٩٦٠/٠٤/٢١ ٢٦٠٠٤٢٤٢٣٠٠٤٤٤

JJ7851478

١٥ م س
تأجير
إيلودا

٢٠٢٢/٠٦ ٢٦٠٠٤٢٤٢٣٠٠٤٤٤

رية منزل
أنشئ
مساعدة

البطاقة سارية حتى ٢٠٢٩/٠٦/٠١

١

07/03/2025

MDT FORM

Name:	Faten Hamama Mahmoud Shehata	MRN:	6246
Age:	65 Y	ID:	26004242300444
Menopausal Status:	postm	Phone:	01110471005
NRS SCORE:	ps 3	Marital Status:	
BMI:		Family History:	
CQ Morbidity:		Reason Of MDT:	mbc triple positive with recent DP

Clinical Data:

BX:

ER PR HER K16:

Sonomammography:

Mets Workup:

History:

postm bilateral breast cancer met to bone rt. er7/8
 pr6/8 her2+ve left er8/8 pr6/8 her23+ rec taxol
 herceptin initial bx 10/2/2025 widespread oss lesions
 pr rec pall rx to left hip for mets 5/5/2026 rd for
 new bone scan
 pr rec 7 cycles herceptin 21 weeks taxol
 bone scan on 18/6/2025 widespread osteoblastic
 and sclerotic osseous lesions

Recommendations:

MDT Outcome & Plan

shift to Tykerb + Xeloda
 Zometa monthly after dental evaluation

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric



ImmunohistoChemistry report

Report Number: m1183

Female :	النوع	فتن حمامه محمد	اسم المريض :
القاهرة :	العنوان	65 Y	السن :
30/07/2025 09:32 :	تاريخ استلام العينة		تاريخ اخذ العينة :
06/08/2025 10:25 :	تاريخ التقرير	مركز صحة المرأة	طالب التحليل :
		مركز صحة المرأة	عناية :

Primary Sample :

Paraffin Block

Test :

ER & PR & HER2 neu & Ki67 (ER&PR: Harvey scoring system -Her2neu : CAP scoring system)

Comment :

Technique :

sample recieved was treated by: - CONFIRM anti-estrogen (ER) (SP1) Rabbit monoclonal primary antibody.- CONFIRM anti-progesterone receptor(PR) (IE2)Rabbit monoclonal primary antibody.

Sample recieved was treated by : -VENTANA anti-HER2/neu (4B5) Rabbit monoclonal primary antibody using DAB Detection kit in ventana benchmark GX / ULTRA .

Sample recieved was treated by :- CONFIRM anti Ki67 (30-8) Rabbit monoclonal primary antibody.

Result :

RIGHT BREAST CORE :- *ER: strong nuclear staining of 60 % of tumour cells

PR: strong nuclear staining of 20 % of tumour cells

HER2/neu protein over expression : discontinuous membrane staining of tumor cells.

Ki67 :positive nuclear staining of 5 % of tumour cells

LEFT BREAST CORE :- *ER: strong nuclear staining of 80 % of tumour cells

PR: strong nuclear staining of 70 % of tumour cells

HER2/neu protein over expression : strong continuous membrane staining of > 10 % of tumor cells.

Ki67 :positive nuclear staining of about 15 % of tumour cells

Referral Lab :

Comment :

Conclusion :

RIGHT BREAST CORE :- *ER: POSITIVE (SCORE 7 / 8)

PR: POSITIVE (SCORE 6 / 8)

HER2/NEU PROTEIN OVER EXPRESSION : NEGATIVE (SCORE 1+)

#Ki67: THE PROLIFERATION INDEX OF THE TUMOUR IS 5 % .

LEFT BREAST CORE :- *ER: POSITIVE (SCORE 8 / 8)

PR: POSITIVE (SCORE 8 / 8)

HER2/NEU PROTEIN OVER EXPRESSION : POSITIVE (SCORE 3+)

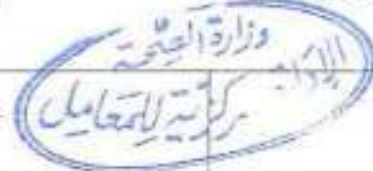
#Ki67: THE PROLIFERATION INDEX OF THE TUMOUR IS ABOUT 15% .

Pathologist:

د/إيهاب رمزي

Lab Director:

د/هريز ماهر





Histopathology report

Report Number: 214

Female :	النوع	فاتن حمامة محمد	سم المريض :
القاهرة :	العنوان	65 Y	لسن :
29/07/2025 10:17 :	تاريخ استلام العينة		تاريخ اخذ العينة :
31/07/2025 11:45 :	تاريخ التقرير	مركز صحة المرأة	طالب التحليل :
		مركز صحة المرأة	عناية :

Primary sample :

- 1- Left breast core biopsy
- 2- Right breast core biopsy

comment :

Gross examination :

- 1 - fragmented cores of greyish white tissue (T . I)
- 2 - fragmented cores of greyish white tissue (T . I)

Microscopic examination :

- 1 - Section examined revealed fibrocollagenous tissue infiltrated by malignant lobular cells arranged in nests & indian files pattern embeded in desmoplastic stroma .
- 2- Section examined revealed fibrocollagenous tissue infiltrated by strands of malignant lobular cells arranged in indian files pattern embeded in desmoplastic stroma.

Referral Lab :

Comment :

Diagnosis :

- 1- LEFT BREAST CORE BIOPSY :- THE PICTURE IS COMPATIBLE WITH INVASIVE LOBULAR CARCINOMA .
- 2- RIGHT BREAST CORE BIOPSY :- THE PICTURE IS COMPATIBLE WITH INVASIVE LOBULAR CARCINOMA .

Pathologist:

DrJehan Aziz

Lab Director:

د.هنا ماهر

Patient ID: 86435	Patient Name English: Faten Hamama Mohamed Shehata
Age: 65 سنة 3 شهر	2025/07/28 11:48:03 AM :Report Date

Women's Health Outreach Program

Bilateral Full field Digital Mammography

Clinical summary: 65-year-old lady with negative family history complaining of right nipple retraction for two months.

Technique

Medio-lateral oblique and CC views High resolution ultrasound

Detailed Findings

Right breast

- Almost entirely fatty breast parenchyma (ACR density a).
- Upper central, middle third, irregular speculated high density mass lesion with surrounding architectural distortion and no suspicious clusters of micro calcifications.
- Thickened skin and coarsened trabeculae.
- **Ultrasound** demonstrated 12 o'clock, 5.6 mm deep to the skin, irregular antiparallel speculated heterogenous hypoechoic solid mass lesion with surrounding echogenic halo, posterior acoustic shadowing and moderate internal vascularity on color Doppler study measuring 40 x 22 mm. Such lesion is seen reaching the nipple through few dilated ducts reaching 7 mm in caliber that are seen with soft tissue material within as well as showing abnormal vascularity within.
- Thickened skin reaching 5.5 mm and coarsened trabeculae.
- Multiple (more than three) discrete pathological enlarged axillary; level 1; LNs are seen the largest measured 24 x 11 mm with 11 focal cortical thickness. Multiple seen at level 2, as well the largest measured 19 x 9 mm and few seen at level 3, largest measured 6 x 6 mm. No pathological supra-clavicular lymph nodes.

Left breast

- Almost entirely fatty breast parenchyma (ACR density a).
- UOQ, middle third, irregular high density mass lesion with micro-lobulated margins and surrounding architectural distortion yet with no suspicious clusters of micro calcifications.
- **Ultrasound** demonstrated 2 o'clock, 4 cm from nipple 1.5 mm deep to the skin, irregular antiparallel speculated heterogenous hypoechoic solid mass lesion with surrounding echogenic halo,

posterior acoustic shadowing and mild internal vascularity on color Doppler study measuring 15 x 6 mm.

- Normal skin/subcutaneous tissue and nipple/areolar complex.
- Two level 1 axillary indeterminate lymph nodes are noted, largest measured 9 x 6 mm with 3 mm cortical thickness. No level 2, 3 or supraclavicular pathological LNs.

Impression:

Right breast (BIRADS 5): Upper central, 12 o'clock locally advanced suspicious mass lesion with multiple ipsilateral enlarged pathological lymph nodes; levels 1,2 & 3. US-guided Tru-cut biopsy was conducted at same setting of the breast lesion.

Left breast (BIRADS 4c): UOQ, 2 o'clock, suspicious solid mass lesion with ipsilateral two indeterminate axillary (level 1) lymph nodes. US-guided Tru-cut biopsy was conducted at same setting of the breast lesion.

Recommendations:

Right breast: histopathological correlation.

Left breast: histopathological correlation.

*Much obliged,
Dr. Amal Hegazi, M.Sc*

Patient's Name: Faten Hamama Mohammed Shehata
Date: 2026-05-05
Referred by: Prof.Dr. CT CAP+C
Hospital: Goustave Roussey International Egypt

**MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST,
ABDOMEN AND PELVIS REVEALED:**

As compared to the last CT examination dated 11/2/2026, the current examination revealed

I- CHEST:

- Stationary course regarding size of the previously noted right breast retro-areolar parenchymal distortion with two nodules largest is measuring 1.2cm and overlying skin thickness with stationary course regarding size and number of the ipsilateral enlarged axillary lymph nodes, the largest still measures about 13.5x9.5 mm as in previous study. **for further sono-mammography**
- Clear left breast and left axilla.
- No pulmonary nodules, masses or cavitary lesions.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Still noted enlarged cardiac size with atherosclerotic calcific foci of the aorta.
- Still noted bilateral elastofibroma dorsi.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- The gall bladder is well distended, average wall thickness with no stones.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or right back pressure changes.
 - Stationary moderate to marked left hydronephrosis and left hydroureter till vesico-ureteric junction with radiodense stone detected ?? attenuated distal end ?? reflux
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.

Patient's Name: Faten Hamama Mohammed Shehata
Date: 2026-05-05
Referred by: Prof.Dr. CT CAP+C
Hospital: Goustave Roussey International Egypt

- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- Multiple pelvic calcific phleboliths are noted.
- No ascites.
- **The examined skeleton**
 - Almost stationary course as regards the multiple lytic and sclerotic bony metastasis along the axial and appendicular skeleton with stationary sclerotic lesions in lower end of sternum, D5 down to L2 , D8, D11 and L3 vertebral collapse and multiple expansile areas/lesions of ribs on both sided being evident on right 6th rib.
 - **For further bone scan.**

Opinion:

- Stationary course since the last study with no denovo lesions.

Dr. Mai Shaaban MD

Dr.Mai Shaaban

Dr.Mai Shaaban

٧٩٤٦

Name : Faten Hamama Mohammed Shehata
Branch : Dokki
Ref.Physician Prof : Goustav Roussey Egypt

Date : 18-06-2026 09:00

Medical No. : 396924

Ins.Company : معاهد مستشفيات

Whole-Body Bone Scintigraphy Report

Indication: Skeletal staging for right breast carcinoma in a 66-year-old female patient; status post chemotherapy and radiotherapy; generalized bone pain.

Procedure: Anterior and posterior whole-body planar imaging after intravenous Tc-99m MDP.

Comparison: Baseline Scan (06/10/2025) vs. Follow-up Scan (2026)

Findings

Skeletal Metastases:

Multifocal, intense abnormal radiotracer uptake is identified throughout the axial and appendicular skeleton, consistent with widespread osseous metastatic disease:

- **Axial Skeleton:** Diffuse involvement of the skull (mixed pattern with prominent occipital uptake), sternum, multiple bilateral ribs, entire spine, and pelvic bones.
- **Appendicular Skeleton:**
 - Bilateral femoral involvement is noted. **New finding:** A prominent, intensely avid lesion in the left femoral mid-shaft.
 - New involvement of the proximal left tibia.
 - Persistent uptake in the right shoulder.

Non-Skeletal Findings:

- Symmetrical, exaggerated periarticular tracer accumulation at both knees and ankles, suggestive of reactive changes or degenerative arthropathy.

Conclusion (considering the prior bone scan)

- **Widespread Progressive Skeletal Metastatic Disease:** Disseminated osteoblastic and mixed sclerotic lesions are stable in distribution but demonstrate **progressive involvement**, with new conspicuous lesions in the left femoral mid-shaft and proximal left tibia.
- **Clinical Correlation:** The prominent left femoral mid-shaft lesion is of particular concern due to an elevated risk of pathological fracture. Orthopedic consultation and consideration of prophylactic intervention or targeted imaging (e.g., CT or MRI) is strongly recommended.

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine

N I L E D I A G N O S T I C C E N T E R

Name : Faten Hamama Mohammed Shehata

Date : 06-10-2025 02:21

Branch : Dokki

Medical No. : 219366

Ref.Physician Prof : Gostaf Roseh Hospital

Tc^{99m} MDPSKELETAL SCINTIGRAPHY

History

- A 65-year-old patient, with history of breast cancer and referred for metastatic workup.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings

- Widespread foci of enhanced tracer uptake are seen at the skull, sternum, multi-levels of the spine, both scapulae, both humeri, bilateral ribs, pelvic bones, both femora, both knees and ankles.
- Degenerative changes are noted at the spine and big joints.

Impression:

- Widespread metastatic osseous lesions as described.

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine

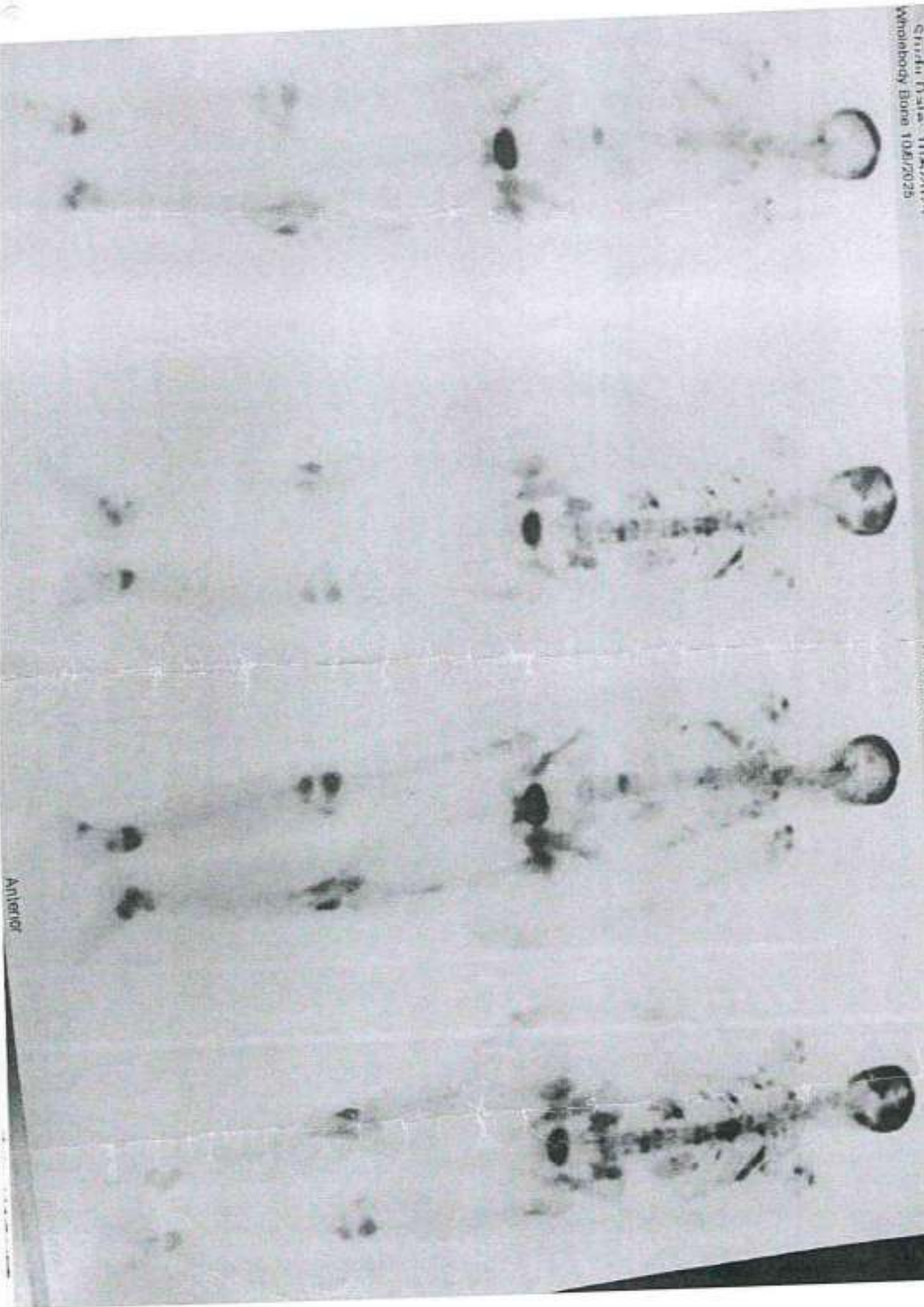
ف. حمادة
الأشعة النووية

1.5

Patient Name: Faten Hamama Mohammed Shehata
DOB: 10/7/1960
Study Date: 10/5/2025
Wholebody Bone 10/5/2025

Faten Hamama Mohammed Shehata DOB: 10/7/1960 ID: 219366 SEX: F
STUDY: Bone Scan STUDY DATE: 10/5/2025 ACCESSION #: 9761691415
Patient ID: 219366
Study Name: Bone Scan

Wholebody Bone 10/5/2025



Anterior

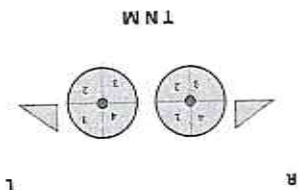
Logo of the Ministry of Health, Kingdom of Saudi Arabia

منظمة الصحة العالمية
وزارة الصحة
08/30/2026

MDT FORM

Name:	Faten Hamama Mohamoud Shehata
Age:	66 Y
Menopausal Sta	
NRS SCORE:	
BMI:	
Gender	
CO Morbidity:	
Surgical History:*	
ECOG Performance Status	0 1 2 3 4
Reason of MDT:	mbc triple positive with query DP
Family History:	
Marital Status:	
Phone:*	01110471005
ID:	26004242300444
MRN:	6246

Clinical Data:



Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4
Right Side				Left Side			

Clinical Staging:

Pathological Staging:

History:

postm bilateral breast cancer met to bone rt er7/8 pr6/8 her2-ve left
er8/8 pr8/8 her23+ rec taxol herceptin initial bs 10/2.025 widespread oss
lesions pt rec pall rth to left hip j sl j cts 5/5/2026 rd for new bone scan
pt rec 7 cycles herceptin 21 weeks taxol
Patient couldn't receive palliative radiotherapy as she is afraid as she took
on the same site before
Last MDT > bone scan on 18/6/2026 widespread osteoblastic and sclerotic
osseous lesions >> shift to Tykerb- Xeloda + Zometa monthly
for reMDT as the CTS show a stationary course while the BS shows query
progressive disease in left midshaft femora + proximal tibia + fibula while
old BS shows uptake in both femoral bones
pt did not receive any xeloda tykerb despite approvals and has continued
herceptin iv to not stop approvals till MDT decision

Sonography:

Biomarker/Molecular

Mets Workup:

Recommendations:

MDT Outcome & Plan

STATIONARY DISEAS FOR NEW TUMOR MARKER CT CAP CONTINUE
HERCPITINE IV / FEMARA
UROLOGY FOR ET HYDRONKROHROSIS
ZOMETA/3 MONTHS
PORTA-CTH
ECHO

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
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د. فاطمة
روسلان
أخصائية أورام



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الكلية الوطنية
لأورام
الجهاز الهضمي

08/30/2026

MDT FORM

Name:	Faten Hamama Mohamoud Shehata	MRN:	6246
Age:	66 Y	ID:	26004242300444
Menopausal Sta		Phone:*	01110471005
NRS SCORE:		Marital Status:	
BMI:		Family History:	
Gender		Reason Of MDT:	
CO Morbidity:			mbc triple positive with query DP
Surgical History:**		ECOG Performance Status	0 00 10 20 30 4

Clinical Data:

R

L

BX: **PR:** 8/8 **HER2:** 3+ **Ki67:**



T N M

History:

postm bilateral breast cancer met to bone rt er7/8 pr6/8 her2-ve left
er8/8 pr8/8 her23+ rec taxol herceptin initial bs 10/2.025 widespread oss
lesions pt rec pall rth to left hip j sj j cts 5/5/2026 rd for new bone scan
pt rec 7 cycles herceptin 21 weeks taxol
Patient couldn't receive palliative radiotherapy as she is afraid as she took
on the same site before

Last MDT > bone scan on 18/6/2026 widespread osteoblastic and sclerotic
osseous lesions -> shift to Tykerb- Xeloda + Zometa monthly

for reMDT as the CTs show a stationary course while the BS shows query
progressive disease in left midshaft femora + proximal tibia + fibula while
old BS shows uptake in both femoral bones

pt did not receive any xeloda tykerb despite approvals and has continued
herceptin iv to not stop approvals till MDT decision

Sonammography:

Biomarker/Molecular

Mets Workup:

Recommendations:

MDT Outcome & Plan

STATIONARY DISEAS FOR NEW TUMOR MARKER CT CAP CONTINUE
HERCPITNE IV / FEMARA
UROLOGY FOR ET HYDRONROHROSIS
ZOMETA/3 MONTHS
PORTA-CTH
ECHO

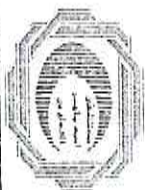
Clinical Staging:

Pathological Staging:

Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د. محمد كمال	د. محمد كمال		



الهيئة العامة للتعليمات والمراكز التعليمية
مستشفى أحمد ماهر التعليمي

The General Organization for Teaching Hospitals
and Institutes

Ahmad Maher Teaching Hospital

To whom it may concern

Mrs./ Mr. عليه السلام عليه السلام, 63 years old

Female male patient presented with diagnosis
4B4 to bone

She He was referred to us for (radical / adjuvant / Neoadjuvant /
palliative) Radiotherapy

She He has received (3D/IMRT) radiotherapy to.....
4B4 - 4B4 It has been

At Dose : 20 Gy/ 5 fx/ 1 w.

Additional (Boost) ; yes/ no at dose Gy/ fx/ w
(sequential / concomitant)

Started: 1/8/2025 30/9/2025

Ended: 5/10/2025 With good tolerance.

Head of department
Dr. Othman William

15/10/23

الهيئة العامة للمستشفيات والمعاهد التعليمية
مستشفى أحمد ماهر التعليمي

The General Organization for Teaching Hospitals
and Institutes

Ahmad Maher Teaching Hospital



To whom it may concern

Mrs. / Mr. Elia فايزة عليا 65 years old
female/ male patient presented with diagnosis
metastatic breast ca to bone

She/ He was referred to us for (radical / adjuvant / Neoadjuvant /
palliative) Radiotherapy

She / He has received (3D/IMRT) radiotherapy to
4 hip + STJ

At Dose : 20 Gy/ 5 fx/ 1 w.

Additional (Boost) ; yes/ no: at dose Gy/ fx/ w
(sequential / concomitant)

Started: 30/9/2023.

Ended: 5/10/2023 With good tolerance.

Head of department
Dr. Hany William
د. هاني رزقي
01000-100000

د. ف. م.
روسل
أ. م. م. م.



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الكلية الوطنية
للطب
الجديد

08/30/2026

MDT FORM

Name:	Faten Hamama Mohamoud Shehata	MRN:	6246
Age:	66 Y	ID:	26004242300444
Menopausal Sta		Phone:*	01110471005
NRS SCORE:		Marital Status:	
BMI:		Family History:	
Gender		Reason Of MDT:	
CO Morbidity:		mbc triple positive with query DP	
Surgical History:**		ECOG Performance Status	
		O 0O 1O 2O 3O 4	

Clinical Data:

R

L

BX: **PR:** 8/8 **HER2:** 3+ **Ki67:**



T N M

History:

postm bilateral breast cancer met to bone rt er7/8 pr6/8 her2-ve left
er8/8 pr8/8 her23+ rec taxol herceptin initial bs 10/2.025 widespread oss
lesions pt rec pall rth to left hip j sj j cts 5/5/2026 rd for new bone scan
pt rec 7 cycles herceptin 21 weeks taxol
Patient couldn't receive palliative radiotherapy as she is afraid as she took
on the same site before

Last MDT > bone scan on 18/6/2026 widespread osteoblastic and sclerotic
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progressive disease in left midshaft femora + proximal tibia + fibula while
old BS shows uptake in both femoral bones

pt did not receive any xeloda tykerb despite approvals and has continued
herceptin iv to not stop approvals till MDT decision

Sonammography:

Biomarker/Molecular

Mets Workup:

Recommendations:

MDT Outcome & Plan

STATIONARY DISEAS FOR NEW TUMOR MARKER CT CAP CONTINUE
HERCPITNE IV / FEMARA
UROLOGY FOR ET HYDRONROHROSIS
ZOMETA/3 MONTHS
PORTA-CTH
ECHO

Clinical Staging:

Pathological Staging:

Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د. محمد كمال	د. محمد كمال		

15/10/23

الهيئة العامة للمستشفيات والمعاهد التعليمية
مستشفى أحمد ماهر التعليمي

The General Organization for Teaching Hospitals
and Institutes

Ahmad Maher Teaching Hospital



To whom it may concern

Mrs. / Mr. Elia فايزة عليا 65 years old
female/ male patient presented with diagnosis
metastatic breast ca to bone

She/ He was referred to us for (radical / adjuvant / Neoadjuvant /
palliative) Radiotherapy

She / He has received (3D/IMRT) radiotherapy to
4 hip + STJ

At Dose : 20 Gy/ 5 fx/ 1 w.

Additional (Boost) ; yes/ no: at dose Gy/ fx/ w
(sequential / concomitant)

Started: 30/9/2023.

Ended: 5/10/2023 With good tolerance.

Head of department
Dr. Hany William
د. هاني رزقي
01000-100000

د. ف. م.
روسل
أ. م. م. م.



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الكلية الوطنية
للطب
الجراحة

08/30/2026

MDT FORM

Name:	Faten Hamama Mohamoud Shehata	MRN:	6246
Age:	66 Y	ID:	26004242300444
Menopausal Sta		Phone:*	01110471005
NRS SCORE:		Marital Status:	
BMI:		Family History:	
Gender		Reason Of MDT:	
CO Morbidity:		mbc triple positive with query DP	
Surgical History:**		ECOG Performance Status	
		O 0O 1O 2O 3O 4	

Clinical Data:

R

L

BX: **PR:** 8/8 **HER2:** 3+ **Ki67:**



T N M

History:

postm bilateral breast cancer met to bone rt er7/8 pr6/8 her2-ve left
er8/8 pr8/8 her23+ rec taxol herceptin initial bs 10/2.025 widespread oss
lesions pt rec pall rth to left hip j sj j cts 5/5/2026 rd for new bone scan
pt rec 7 cycles herceptin 21 weeks taxol
Patient couldn't receive palliative radiotherapy as she is afraid as she took
on the same site before

Last MDT > bone scan on 18/6/2026 widespread osteoblastic and sclerotic
osseous lesions >> shift to Tykerb- Xeloda + Zometa monthly

for reMDT as the CTs show a stationary course while the BS shows query
progressive disease in left midshaft femora + proximal tibia + fibula while
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pt did not receive any xeloda tykerb despite approvals and has continued
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Sonammography:

Biomarker/Molecular

Mets Workup:

Recommendations:

MDT Outcome & Plan

STATIONARY DISEAS FOR NEW TUMOR MARKER CT CAP CONTINUE
HERCPITNE IV / FEMARA
UROLOGY FOR ET HYDRONROHROSIS
ZOMETA/3 MONTHS
PORTA-CTH
ECHO

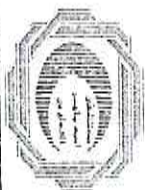
Clinical Staging:

Pathological Staging:

Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د. محمد كمال	د. محمد كمال		



الهيئة العامة للتعليمات والمراكز التعليمية
مستشفى أحمد ماهر التعليمي

The General Organization for Teaching Hospitals
and Institutes

Ahmad Maher Teaching Hospital

To whom it may concern

Mrs./ Mr. Dr. Laila Dr. Laila, 63 years old

Female male patient presented with diagnosis
4B4 to bone

She He was referred to us for (radical / adjuvant / Neoadjuvant /
palliative) Radiotherapy

She He has received (3D / IMRT) radiotherapy to
At 4B4 - 4B4 It has been

At Dose : 20 Gy / 5 fx / 1 w.

Additional (Boost) ; yes/ no at dose Gy / fx / w
(sequential / concomitant)

Started: 1/8/2025 30/9/2025

Ended: 5/10/2025 With good tolerance.

Head of department
Dr. Othman William

15/10/23

الهيئة العامة للمستشفيات والمعاهد التعليمية
مستشفى أحمد ماهر التعليمي

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and Institutes

Ahmad Maher Teaching Hospital



To whom it may concern

Mrs. / Mr. Elia فانيلة 65 years old
female/ male patient presented with diagnosis
metastatic breast ca to bone

She/ He was referred to us for (radical / adjuvant / Neoadjuvant /
palliative) Radiotherapy

She / He has received (3D/IMRT) radiotherapy to
4 hip + STJ

At Dose : 20 Gy/ 5 fx/ 1 w.

Additional (Boost) : yes/ no: at dose Gy/ fx/ w
(sequential / concomitant)

Started: 30/9/2023.

Ended: 5/10/2023 With good tolerance.

Head of department
Dr. Hany William
د. هاني رزقي
01000-100000

د. فاطمة
روسلان
أخصائية أورام



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الكلية الوطنية
للأورام
مصر

08/30/2026

MDT FORM

Name:	Faten Hamama Mohamoud Shehata	MRN:	6246
Age:	66 Y	ID:	26004242300444
Menopausal Sta		Phone:*	01110471005
NRS SCORE:		Marital Status:	
BMI:		Family History:	
Gender		Reason Of MDT:	
CO Morbidity:		mbc triple positive with query DP	
Surgical History:**		ECOG Performance Status	
		0 00 10 20 30 4	

Clinical Data:

R

L

BX: **PR:** 8/8 **HER2:** 3+ **Ki67:**



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Biomarker/Molecular

Mets Workup:

Recommendations:

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HERCPITNE IV / FEMARA
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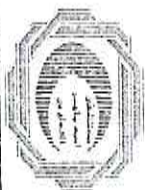
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SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د. محمد كمال	د. محمد كمال		



الهيئة العامة للتعليمات والمراكز التعليمية
مستشفى أحمد ماهر التعليمي

The General Organization for Teaching Hospitals
and Institutes

Ahmad Maher Teaching Hospital

To whom it may concern

Mrs./ Mr. Dr. Laila Dr. Laila, 63 years old

Female male patient presented with diagnosis
MBK to bone

She He was referred to us for (radical / adjuvant / Neoadjuvant /
palliative) Radiotherapy

She He has received (3D/IMRT) radiotherapy to.....
At the hip joint

At Dose : 20 Gy/ 5 fx/ 1 w.

Additional (Boost) ; yes/ no at dose Gy/ fx/ w
(sequential / concomitant)

Started: 1/8/2025 30/9/2025

Ended: 5/10/2025 With good tolerance.

Head of department
Dr. Hany William



البيئة العامة للمستشفيات والمصادر التعليمية
مستشفى احمد ماهر التليسي

The General Organization for Teaching Hospitals
and Institutes

Ahmad Maher Teaching Hospital

To whom it may concern

Mrs. / Mr. *أحمد محمد*, 65 years old
female/ male patient presented with diagnosis

Metastatic breast $\rightarrow$ to bone

She/He was referred to us for (radical / adjuvant / Neoadjuvant / palliative) Radiotherapy

She / He has received (3D / IMRT) radiotherapy to

4 hip - S1J

At Dose: 20 Gy / 5 fx / 1 w.

Additional (Boost) ; yest no. at dose.....Gy/.....fx/.....w
(sequential / concomitant)

Started: 30.09.2025.

Ended: \$ 16/202 \$ With good tolerance.

Head of department

Dr. Henry William

2000-10-01

د. ف. م.
روسل
أ. م. م. م.



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الكلية الوطنية
للطب
الجراحة
والتوليد

08/30/2026

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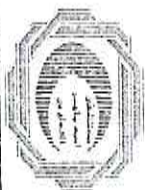
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MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د. محمد كمال	د. محمد كمال		



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مستشفى أحمد ماهر التعليمي

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Started: 1/8/2025 30/9/2025

Ended: 5/10/2025 With good tolerance.

Head of department
Dr. Hany William



البيئة العامة للمستشفيات والمصادر التعليمية
مستشفى احمد ماهر التليسي

**The General Organization for Teaching Hospitals
and Institutes**

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To whom it may concern

Mrs. / Mr. *أحمد محمد*, 65 years old
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(sequential / concomitant)

Started: 30.9.2025.

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Head of department

Dr. Henry William

6.50-10.00